



HEXAMED

DIAGNOSTICS & SPECIALTY CLINICS

Your Right to Right Healthcare...

Laboratory | Radiology | Polyclinic | Pharmacy

Barcode No	87943906	Lab No	16952503290004
Patient Name	Ms.NAINA	Reg Date	29/Mar/2025 02:48PM
Age/Sex	21 YRS/Female	Sample Coll. Date	29/Mar/2025 01:54 PM
Referred By	SELF	Sample Rec.Date	30/Mar/2025 08:11 AM
		Report Date	30/Mar/2025 12:10PM

IMMUNOASSAY

Test Name With Methodology

Result

Unit

Biological Ref.Interval

TTG- IgA (Tissue Transglutaminase IgA Antibody)

Tissue Transglutaminase Ab IgA
Serum, ELISA

0.43

U/mL

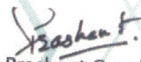
<4.0 Negative
4-10 Weak Positive
>10 Positive

Comments:

Useful for Evaluating patients suspected of having celiac disease, as Screening test for dermatitis herpetiformis, in conjunction with endomysial antibody test & Monitoring adherence to gluten-free diet in patients with dermatitis herpetiformis and celiac disease. A definitive diagnosis of celiac disease requires a jejunal biopsy demonstrating villous atrophy. Those individuals with positive laboratory results should be referred for small intestinal biopsy to confirm the diagnosis. If patients strictly adhere to a gluten-free diet, the unit value of IgA-anti-tTG should begin to decrease within 6 to 12 months of onset of dietary therapy.

CAUTIONS

This test should not be solely relied upon to establish a diagnosis of celiac disease. It should be used to identify patients who have an increased probability of having celiac disease and in whom a small intestinal biopsy is recommended. Affected individuals who have been on a gluten-free diet prior to testing may have a negative result. For individuals who test negative, IgA deficiency should be considered. If total IgA is normal and tissue transglutaminase (tTG)-IgA is negative, there is a low probability of the patient having celiac disease and a biopsy may not be necessary. If serology is negative or there is substantial clinical doubt remaining, then further investigation should be performed with endoscopy and bowel biopsy. This is especially important in patients with frank malabsorptive symptoms since many syndromes can mimic celiac disease. For the patient with frank malabsorptive symptoms, bowel biopsy should be performed regardless of serologic test results.


Dr. Prashant Goyal (DCP)
(Director & Chief Pathologist)
Reg. No. DMC-53016

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Corporate Office: Flat no. 203, Sagacity heights, Plot No. 27, Kavuri Hills Road, CBI Colony, Madhapur, Telangana 500033

Jammu Address: 25-270/6, Main Bypass Road, NH-1A, Channi Rama, Jammu, J & K 180015

✉ feedback@hexamedhealthcare.com @ enquiry@hexamedhealthcare.com 📞 support@hexamedhealthcare.com

📞 +91 95419 01145

📞 1800 212 6547

🌐 www.hexamedhealthcare.com

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